

Response to HHS Health Sector AI RFI: Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care (RIN 0955-AA13)

To: Department of Health and Human Services (HHS)

From: Citrix Systems, Inc., a Cloud Software Group company

Date: February 23, 2026

Subject: Comment on RFI: Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care (RIN 0955-AA13)

I. Executive Summary: The Infrastructure for AI Readiness

While much of the healthcare industry's focus remains on the development of AI-powered applications, Citrix identifies the primary bottleneck to adoption as the deployment and delivery layer. In a healthcare environment characterized by shared workstations, mobile clinicians, and strict HIPAA requirements, an AI tool is only as effective as its availability at the point of care.

Citrix provides an industry leading platform that serves as a digital foundation for many healthcare organizations in support of delivering secure, connected healthcare services. With a footprint that includes a majority of the largest healthcare organizations in the United States and over 4,900 healthcare customers worldwide, our role is to provide the governance layer and platform that allows healthcare organizations to transition from AI experimentation to large-scale clinical use without compromising security or productivity. Our platform also helps remove friction at the bedside to ensure that technology works invisibly in the background, allowing care teams to stay focused on patients.

II. Response to Specific RFI Questions (1,3,5,6)

Q1: What are the primary barriers to the adoption of AI in clinical settings?

Citrix identifies the primary barrier to AI adoption as a deployment gap — the technical and operational friction that prevents an AI insight from reaching a clinician at the point of care.

To overcome this, HHS should address the following structural barriers:

- **The "Last Mile" delivery problem:** High-performing AI models have low adoption in clinical settings because they are not integrated into the roaming workflows of a hospital. In environments dominated by shared workstations, a clinician cannot afford the "cognitive tax" of logging into a separate, disconnected AI application.